



Infection Prevention and Management Manual

Infectious Disease Summary Table Guideline

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

This table lists common infectious diseases and the minimum infection prevention and management precautions required. The majority of infectious diseases requiring transmission-based precautions are listed here. Individual cases and diseases not outlined in this list may be discussed with Infection Prevention and Control Unit or the on-call Clinical Microbiologist. For further information refer to SCGOPHCG IP&C Manual: Standard and Transmission Based Precautions Policy.

The staff exclusions apply to both suspected and confirmed cases and only during the infectious period. For further information on staff restrictions, including pregnant staff, refer to SCGOPHCG IP&C Manual: Healthcare Worker Health and Immunisation Policy.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		



Table 1: Type and Duration of Precautions for Specific Infectious Diseases

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Acquired Immune Deficiency Syndrome (AIDS)	See 'Human Immunodeficiency Virus (HIV)'				
Amoebic dysentery	Acute enteritis caused by the protozoan parasite <i>Entamoeba histolytica</i> . Transmission usually from ingestion of contaminated food or water. Limited person-to-person transmission via faecal-oral route.	CONTACT PRECAUTIONS if the patient has diarrhoea. Otherwise STANDARD PRECAUTIONS	Until symptoms resolve.		NO
Adenovirus	A common cause of viral upper respiratory tract infection. Transmitted by contact with respiratory droplets.	DROPLET PRECAUTIONS		Refer to SCGOPHCG Infection Prevention and Management Influenza-like Illness (ILI) Policy	NO
Avian influenza	Severe respiratory tract disease caused by non-endemic avian subtypes of influenza A virus. Usually transmitted by direct or indirect contact with infected birds. Limited person-to-person transmission.	AIRBORNE PRECAUTIONS		Refer to SCGOPHCG Infection Prevention and Management Respiratory Viruses (Non-Endemic) Policy. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant staff should NOT attend patients with avian influenza.

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
<i>Campylobacter</i>	Acute bacterial enteritis. Transmission usually via ingestion of contaminated food. Limited person-to-person transmission via faecal-oral route.	CONTACT PRECAUTIONS	Until symptoms resolve.		NO
<i>Candida auris</i>	Refer to SCGOPHCG Infection Prevention and Management Multi-Resistant Organism Policy.				NO
Carbapenem-resistant <i>Enterobacteriaceae</i> (CRE)	Refer to SCGOPHCG Infection Prevention and Management Multi-Resistant Organism Policy.				NO
Chickenpox (varicella)	Chickenpox is an acute generalised viral infection caused by varicella-zoster virus (VZV) and is highly contagious. Transmission occurs by contact with respiratory aerosols or vesicle fluid. Infection in pregnancy is associated with adverse pregnancy outcomes.	AIRBORNE PRECAUTIONS	Until all lesions are dry and crusted.	Post-exposure prophylaxis should be considered for non-immune staff or patients who are exposed. Refer to SCGOPHCG Varicella-Zoster Virus Policy. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant staff and non-immune staff should NOT attend patients with chickenpox.
Chlamydomphila pneumonia	Community-acquired pneumonia caused by <i>Chlamydomphila pneumoniae</i> . Transmitted by respiratory droplets.	DROPLET PRECAUTIONS	Until symptoms resolve.		NO
Cholera	Acute bacterial enteritis caused by <i>Vibrio cholerae</i> . Transmission is via the faecal-oral route.	CONTACT PRECAUTIONS	Until symptoms resolve.	Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
<i>Clostridioides difficile</i> (formerly <i>Clostridium difficile</i>)	Bacterial enteritis usually associated with antibiotic use. Transmission by faecal-oral route via contact with faeces or bacterial spores in the environment.	CONTACT PRECAUTIONS	Until no diarrhoea for 48hrs.	Refer to SCGOPHCG Infection Prevention and Management <i>Clostridioides</i> (<i>Clostridium</i>) <i>difficile</i> Policy.	NO
Conjunctivitis – bacterial	Transmitted by direct or indirect contact with respiratory secretions or conjunctival discharge of infected persons.	STANDARD PRECAUTIONS			NO
Conjunctivitis – viral	Common causes are adenovirus, enterovirus, Coxsackievirus A24. Transmitted by direct or indirect contact with respiratory secretions or conjunctival discharge of infected persons.	CONTACT PRECAUTIONS	Until conjunctivitis resolved.		NO
Coronavirus (seasonal) (coronavirus HKU1, NL63, 229E and OC43)	Seasonal coronaviruses usually cause mild respiratory illness such as common cold symptoms	DROPLET PRECAUTIONS		Refer to SCGOPHCG Infection Prevention and Management Influenza-like Illness (ILI) Policy	NO
COVID-19	Refer to SCGOPHCG COVID-19 Policy.				Pregnant staff should NOT attend a confirmed or suspect case of COVID-19.

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Creutzfeldt–Jakob Disease (CJD)	Rare neurodegenerative disorder caused by a prion. Person-to-person transmission does not occur under normal circumstances. Iatrogenic transmission has occurred from direct or indirect exposure to CNS tissues (e.g. dura mater grafts, contaminated neurosurgical instruments).	STANDARD PRECAUTIONS in ward areas.		Additional precautions may be required in the operating theatre. Refer to SCGOPHCG Infection Prevention and Management CJD Policy. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	NO
Cryptosporidiosis	Acute parasitic enteritis. Transmitted via the faecal-oral route.	CONTACT PRECAUTIONS	Until symptoms resolve.		NO
Cytomegalovirus (CMV)	Common viral infection. Infection in pregnancy associated with adverse pregnancy outcomes. Transmission by mucosal contact with infected secretions or urine.	STANDARD PRECAUTIONS		Transmission within the healthcare setting is uncommon. The risk of acquiring infection by seronegative staff is low.	Pregnant staff should NOT attend patients with active CMV disease.
Dengue Fever	Mosquito-borne viral infection usually acquired outside Australia. No person-to-person transmission under normal circumstances.	STANDARD PRECAUTIONS			NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Diarrhoea	Diarrhoea may be infectious (viral, bacterial, parasitic) or non-infectious. Infectious diarrhoea is usually transmitted by the faecal-oral route. See separate sections for specific causes.	CONTACT PRECAUTIONS for infectious or unexplained diarrhoea.	If a microbiological diagnosis is made, see the relevant sections for duration of precautions.	For unexplained diarrhoea, faeces should be submitted for microbiological testing. If microbiological testing is negative, contact precautions can cease when symptoms resolve or if a non-infectious cause of diarrhoea is identified.	NO
Diphtheria	Vaccine-preventable acute bacterial disease involving respiratory tract or skin, caused by toxigenic strains of <i>Corynebacterium</i> . Transmission by contact with skin lesions or respiratory secretions.	CONTACT PRECAUTIONS if the patient has skin lesions. DROPLET PRECAUTIONS if the patient has respiratory disease.	Until antimicrobial treatment completed and culture negative.		NO
E. coli	See 'Shiga-toxigenic E. coli (STEC)'				
Epiglottitis	Epiglottitis has numerous infective and non-infective causes. Invasive meningococcal disease (IMD) can occasionally present with epiglottitis.	DROPLET PRECAUTIONS	Until 24 hours after commencement of antimicrobial treatment.	If IMD is confirmed, refer to SCGOPHCG Infection Prevention and Management Meningococcal Disease Policy.	NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Gastroenteritis (viral)	Includes norovirus, rotavirus, adenovirus 40/41. Person-to-person transmission is via the faecal-oral route, e.g. direct contact with faeces or vomit or indirect contact via contaminated objects or environmental surfaces.	CONTACT PRECAUTIONS	For 48 hours after symptom resolution.	If patient is immunosuppressed, the duration of precautions should be assessed on an individual basis in conjunction with Infection Prevention and Management. Inform the Infection Prevention and Management Unit if there are more than two patients with unexplained diarrhoea on one ward area.	NO
Giardiasis	Parasitic enteritis. May be asymptomatic. Transmission is via the faecal-oral route or contact with contaminated water.	CONTACT PRECAUTIONS if the patient has diarrhoea. Otherwise STANDARD PRECAUTIONS.	Until symptoms resolve.		NO
Genetically Modified Organisms	Certain novel therapies involve administration of genetically-modified replicating agents...	Assessed on a case by case basis in discussion with Infection Prevention and Management Unit.		Please inform the Infection Prevention and Management Unit about any patient receiving such therapies.	NO
Glandular Fever	See 'Infectious Mononucleosis'				
Group A Streptococci (invasive)	Invasive disease caused by group A streptococci (<i>S. pyogenes</i>). Transmitted person-to-person via respiratory droplets.	DROPLET PRECAUTIONS	Until 24h after commencement of treatment with effective antibiotics.		NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Hand, Foot and Mouth Disease	Viral infection caused by enterovirus. Transmission by contact with blister fluid, respiratory secretions or faeces.	CONTACT PRECAUTIONS	Until all lesions dry and crusted.		NO
Hepatitis A	Vaccine-preventable viral hepatitis. Transmission is via the faecal-oral route from person to person, or from contaminated food or water.	CONTACT PRECAUTIONS	Until one week after onset of jaundice or dark urine or two weeks after onset of prodromal symptoms.	Post-exposure prophylaxis with vaccine or normal human immunoglobulin may be considered for non-immune close contacts. Contact Infection Prevention and Management or a Clinical Microbiologist for advice.	NO
Hepatitis B	Vaccine-preventable viral hepatitis that may be acute or chronic. Transmitted by percutaneous or mucosal exposure to infected blood or body fluids.	STANDARD PRECAUTIONS		Health care workers who are at risk of exposure should be immunised. Post-exposure prophylaxis should be considered for non-immune staff and patients who are exposed. Contact a Clinical Microbiologist for advice.	NO
Hepatitis C	Viral hepatitis that may be acute or chronic. Most commonly transmitted by blood to blood contact. Sexual transmission is rare.	STANDARD PRECAUTIONS			NO
Hepatitis E	Viral hepatitis rare in Australia. Transmission is via the faecal-oral route, usually by contaminated food or water.	CONTACT PRECAUTIONS	Until two weeks after onset of jaundice.		NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Herpes Simplex Virus (HSV)	Common viral infection usually spread by contact with infected lesions or secretions.	CONTACT PRECAUTIONS Only if severe mucocutaneous or disseminated disease. Otherwise STANDARD PRECAUTIONS	Until lesions dry and crusted.		NO
Human Immunodeficiency Virus (HIV)	A viral infection resulting in progressive immune deficiency. Transmitted by percutaneous or mucosal exposure to infected blood or body fluids.	STANDARD PRECAUTIONS			NO
Human metapneumovirus (HMPV)	See 'Respiratory viruses'				
Infectious mononucleosis (Glandular fever)	Generalised viral disease caused by Epstein-Barr virus (EBV). Transmission is via direct mucosal contact with saliva.	STANDARD PRECAUTIONS			NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Influenza	Acute viral respiratory infection. Transmission is via direct or indirect contact with respiratory secretions via contaminated equipment or the environment. Immunosuppressed or severely unwell patients may be infectious for prolonged periods.	DROPLET PRECAUTIONS		If patient is immunocompromised or a critical care patient e.g. ICU, G73, G63, the duration of precautions should be assessed on an individual basis in conjunction with the Infection Prevention and Management Unit. Refer to SCGOPHCG Infection Prevention and Management Influenza-Like Illness (ILI) Policy.	Pregnant staff and staff at risk of severe influenza should NOT attend patients with influenza.
Influenza-like illness (ILI)	Influenza-like illness (ILI) is an acute illness caused by several viruses that are known to be transmitted via respiratory droplets, including influenza, parainfluenza, respiratory syncytial virus human metapneumovirus, rhinovirus, adenovirus and seasonal coronaviruses (coronavirus 229E, HKU1, OC43, NL63).			Refer to SCGOPHCG Infection Prevention and Management Influenza-Like Illness (ILI) Policy.	Pregnant staff and staff at risk of severe influenza should NOT attend patients with influenza.
Impetigo (School sores)	Common bacterial infection of the skin caused by staphylococcal or streptococcal bacteria. Transmission is via contact with skin lesions.	CONTACT PRECAUTIONS	For 24hrs after commencing treatment or, if untreated, until lesions dry.		NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Legionellosis (Legionnaire's disease)	An acute bacterial disease of the respiratory tract caused by <i>Legionella pneumophila</i> or <i>L. longbeachae</i> . Transmitted by exposure to contaminated environmental sources (e.g. water, soil). No person-to-person transmission.	STANDARD PRECAUTIONS		Notify Infection Prevention and Management Unit of any suspected hospital-acquired case.	NO
Leprosy (Hansen's Disease)	Chronic bacterial disease of the skin, nerves or upper airway. Transmitted by contact with nasal discharge.	STANDARD PRECAUTIONS			NO
Leptospirosis	A bacterial zoonotic disease. Transmitted by direct or indirect contact secretions from infected animals.	STANDARD PRECAUTIONS			NO
Lice	Infestation by <i>Pediculus humanus</i> . Usually transmitted via infested clothing. Head lice are more readily transmissible person-to-person.	CONTACT PRECAUTIONS only for head lice. Otherwise STANDARD PRECAUTIONS.	Until 24h after treatment commenced.	Refer to SCGH Infection Prevention and Management Scabies, Fleas and Lice Guideline. Wear gown and gloves while removing clothing from infested patients.	NO
Listeriosis	Acute enteric infection which may disseminate. Transmission by ingestion of contaminated food.	STANDARD PRECAUTIONS			NO
Malaria	Mosquito-borne parasitic infection.	STANDARD PRECAUTIONS			NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Measles	Highly contagious, vaccine-preventable, acute generalised viral disease. Transmission is by the airborne route.	AIRBORNE PRECAUTIONS	Until 4 days after rash appears.	Post-exposure prophylaxis should be considered for non-immune staff or patients who are exposed. Refer to SCGOPHCG Infection Prevention and Management Measles Policy for further information. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant staff and non-immune staff should NOT attend patients with measles.
Melioidosis	A bacterial disease caused by <i>Burkholderia pseudomallei</i> . Transmission by contact with contaminated soil or water.	STANDARD PRECAUTIONS			NO
Meningitis (unknown aetiology)	See 'Meningococcal disease (invasive)'.				
Meningitis/ encephalitis (viral)	E.g. Herpes simplex virus (HSV), enterovirus.	STANDARD PRECAUTIONS			NO
Meningococcal disease (invasive)	Acute bacterial infection caused by <i>Neisseria meningitidis</i> . Transmission by direct contact with respiratory secretions.	DROPLET PRECAUTIONS	For 24hrs after beginning appropriate antibiotic treatment.	Refer to SCGOPHCG Infection Prevention and Management Meningococcal Disease Policy. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Middle East Respiratory Syndrome (MERS-CoV)	A severe non-endemic zoonotic respiratory viral illness. Usually transmitted by contact with respiratory secretions from infected animals (e.g. camels). Limited person-to-person transmission occurs.	AIRBORNE PRECAUTIONS	On advice of the Infection Prevention and Management Unit or a Clinical Microbiologist.	Refer to SCGOPHCG Infection Prevention and Management Policy Respiratory Viruses (non-endemic). Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant staff and staff at risk of severe disease should NOT attend patients with MERS-CoV.
Mpox (formerly Monkeypox)	Rare disease caused by the Monkeypox virus. Incubation period can range from 5-21 days. A maculopapular rash appears typically 1-5 days after a prodrome period. Transmission can occur through contact with lesions and body fluids, including sexual contact and fomites.	CONTACT and AIRBORNE PRECAUTIONS	Until all lesions are dry, have scabbed over and fallen off, leaving fresh intact skin.	Refer to SCGOPHCG Infection Prevention and Management - Mpox Policy	No
MRSA (methicillin-resistant <i>Staphylococcus aureus</i>)	Refer to SCGOPHCG Infection Prevention and Management Multi-Resistant Organism Policy.				No
Mumps	Mumps is an acute viral infection. Transmitted by contact with infected respiratory secretions.	DROPLET PRECAUTIONS	Until 9 days after onset of symptoms.	Exposed non-immune people should be considered infectious from 12-25 th day after exposure, with or without symptoms. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Non-immune staff should NOT attend patients with mumps.

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Mycoplasma pneumonia	Community-acquired pneumonia caused by <i>Mycoplasma pneumoniae</i> . Transmitted by respiratory droplets.	DROPLET PRECAUTIONS	Until symptoms resolve.		No
Nipah virus	A severe non-endemic zoonotic viral infection causing respiratory disease and/or encephalitis. Limited person-to-person transmission occurs	CONTACT and AIRBORNE PRECAUTIONS	As advised by Clinical Microbiology.	Refer to SCGOPHCG Infection Prevention and Control 'Respiratory Viruses (Non-Endemic)' policy. Notify Infection Prevention and Control Unit of all suspected or confirmed cases.	Pregnant staff should NOT attend patients with suspected or confirmed Nipah virus.
Norovirus	See 'Gastroenteritis (viral)'				
Parainfluenza	See 'Respiratory viruses'	DROPLET PRECAUTIONS			
Parvovirus B19	A common self-limiting viral disease. Infection in pregnancy is associated with adverse pregnancy outcomes. Transmitted by contact with respiratory droplets. Immunosuppressed patients may develop chronic infection and be infectious for long periods.	DROPLET PRECAUTIONS	Once rash or arthralgia have developed, immunocompetent patients are no longer infectious. For patients without rash/arthralgia or who are immunosuppressed, duration of precautions should be assessed on an individual basis in conjunction with Infection Prevention and Management.		Pregnant staff should NOT attend immunocompetent patients during the acute infectious period, or immunocompromised patients with chronic parvovirus infection.

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Pertussis (Whooping cough)	Pertussis is an acute bacterial respiratory disease. Transmitted by contact with respiratory secretions.	DROPLET PRECAUTIONS	Until patient has received 5 days of antibiotic therapy (e.g. azithromycin) or, if untreated, 3 weeks after symptom onset.	Post-exposure prophylaxis should be considered for high-risk contacts. Notify Infection Prevention and Management Unit of all suspected or confirmed cases. ** Standard precautions apply to detections of <i>Bordetella parapertussis</i>.	Pregnant staff should NOT attend patients with pertussis. Staff should avoid contact if they have not been vaccinated.
Pneumococcal disease (invasive)	Bacterial infection with <i>Streptococcus pneumoniae</i> . Pneumonia is the most common manifestation. Asymptomatic carriage in the upper respiratory tract is common.	STANDARD PRECAUTIONS			NO
<i>Pneumocystis jiroveci</i> pneumonia (PJP)	An opportunistic infection caused by the fungus <i>P. jiroveci</i> . Transmitted by contact with respiratory secretions or airborne route. Hospital outbreaks among immunosuppressed patients have been reported.	STANDARD PRECAUTIONS		A single room is preferred for patients on G73 on treatment for <i>Pneumocystis</i> pneumonia until 7 days after treatment commenced.	Staff at risk of <i>Pneumocystis</i> pneumonia should NOT attend patients with confirmed infection.
Poliomyelitis	Vaccine-preventable viral disease now extremely uncommon in Australia. Transmitted by contact with faeces or oral secretions.	CONTACT PRECAUTIONS	Duration of precautions should be determined in conjunction with Infection Prevention and Management.		NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Rabies	Rabies is a zoonotic viral infection. Transmitted by percutaneous or mucosal exposure to infected animals (e.g. animal bite).	STANDARD PRECAUTIONS			NO
Respiratory viruses (including respiratory syncytial virus (RSV), parainfluenza virus, human metapneumovirus (HMPV))	Acute viral illness usually causing an influenza-like illness. Transmitted by contact with respiratory droplets. Immunosuppressed or severely unwell patients may be infectious for prolonged periods.	DROPLET PRECAUTIONS		If patient is immunocompromised or a critical care patient e.g. ICU, G73, G63, the duration of precautions should be assessed on an individual basis in conjunction with the Infection Prevention and Management Unit. Refer to SCGOPHCG Infection Prevention and Management Influenza-Like Illness (ILI) Policy.	Pregnant staff and staff at risk of severe disease should NOT attend patients with influenza-like illness.
Rotavirus	See 'Gastroenteritis (viral)'				
Rhinovirus	A common cause of viral upper respiratory tract infection. Transmitted by contact with respiratory droplets.	DROPLET PRECAUTIONS		Refer to SCGOPHCG Infection Prevention and Management Influenza-like Illness (ILI) Policy	NO
RSV (Respiratory syncytial virus)	See 'Respiratory viruses'	DROPLET PRECAUTIONS		Refer to SCGOPHCG Infection Prevention and Management Influenza-like Illness (ILI) Policy	NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Rubella	Vaccine-preventable viral infection. Infection in pregnancy is associated with adverse pregnancy outcomes. Transmitted by contact with respiratory droplets.	DROPLET PRECAUTIONS	Until 7 days after onset of rash.	Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant staff and non-immune staff should NOT attend patients with rubella.
Salmonella	Acute bacterial enteritis. Transmitted by ingestion of contaminated food and faecal-oral route. Limited person-to-person transmission.	CONTACT PRECAUTIONS	Until symptoms resolve.		NO
Scabies	Infestation with <i>Sarcoptes scabiei</i> . Transmitted by direct or indirect contact with infested persons.	CONTACT PRECAUTIONS	Non crusted: Until 24h after treatment commenced. Crusted: Until all treatment complete and at least 10 days have passed since treatment initiation	Refer to SCGOPHCG Infection Prevention and Management Scabies, Fleas and Lice Guideline.	NO
Scarlet Fever	A bacterial disease caused by Group A streptococci (<i>S. pyogenes</i>). Transmission is via direct contact with respiratory secretions or droplet spread.	DROPLET PRECAUTIONS	Until 24 hours after commencement of effective antibiotic therapy.	See also 'Group A Streptococci (invasive)'	NO

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Severe Acute Respiratory Syndrome (SARS-CoV)	A rare, non-endemic viral infection transmitted by contact with respiratory secretions.	AIRBORNE PRECAUTIONS	On the advice of the Infection Prevention and Management Unit or a Clinical Microbiologist.	Refer to SCGH Infection Prevention and Management Policy Respiratory Viruses (non-endemic). Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant staff and staff at risk of severe disease should NOT attend patients with SARS-CoV.
Shiga-toxigenic E. coli (STEC)	Bacterial cause of dysentery. Transmitted by faecal-oral route, usually via contaminated food.	CONTACT PRECAUTIONS Only if diarrhoea or incontinence. Otherwise STANDARD PRECAUTIONS.	Until symptoms resolve.	No transmission-based precautions required for non-STEC E. coli.	NO
Shigella	An acute bacterial enteritis. Person-to-person transmission by the faecal-oral route.	CONTACT PRECAUTIONS	Until symptoms resolve.		NO
Shingles (herpes zoster)	Cutaneous reactivation of latent VZV infection. May transmit VZV by exposure to vesicle fluid or respiratory secretions. Disseminated shingles is more infectious.	CONTACT PRECAUTIONS for patients with localised shingles. AIRBORNE PRECAUTIONS for patients with disseminated shingles.	Until all lesions dry and crusted.	Post-exposure prophylaxis should be considered for non-immune staff or patients who are exposed. Refer to SCGOPHCG Infection Prevention and Management Varicella-Zoster Virus Policy.	Pregnant staff and non-immune staff should NOT attend patients with shingles.

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Tuberculosis (pulmonary)	Bacterial disease of the lower respiratory tract. Respiratory disease transmitted by the airborne route.	AIRBORNE PRECAUTIONS	Consult with Respiratory Physician or Infection Prevention and Management Unit.	Refer to SCGOPHCG Infection Prevention and Management Tuberculosis Policy. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	Pregnant HCWs should NOT attend patients with suspected or confirmed pulmonary tuberculosis.
Tuberculosis (extra-pulmonary)	Extra-pulmonary tuberculosis is not normally transmissible unless there is a discharging lesion.	CONTACT PRECAUTIONS if the patient has a discharging lesion. Otherwise STANDARD PRECAUTIONS .	Consult with Infection Prevention and Management Unit.	Refer to SCGOPHCG Infection Prevention and Management Tuberculosis Policy. Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	NO
Typhoid	Typhoid fever is caused by <i>Salmonella enterica</i> serotype Typhi. Transmission is via the faecal-oral route.	CONTACT PRECAUTIONS	For the duration of illness. Consult with Infection Prevention and Management Unit if required.		NO
Varicella zoster virus (VZV)	See 'Chickenpox' or 'Shingles'				
VRE (vancomycin-resistant <i>Enterococcus</i>)	Refer to SCGOPHCG Infection Prevention and Management Multi-Resistant Organism Policy.				
Viral Haemorrhagic Fever (VHF) Includes Ebola, Marburg, Crimean-Congo haemorrhagic fever, Lassa fever.	Refer to SCGOPHCG Infection Prevention and Management Viral Haemorrhagic Fever Policy.			Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	

DISEASE/ ORGANISM	DESCRIPTION	TRANSMISSION- BASED PRECAUTIONS	DURATION OF PRECAUTIONS	NOTES	STAFF EXCLUSIONS
Whooping cough	See 'Pertussis'				
<i>Yersinia pestis</i> (plague)	A rare zoonotic bacterial disease. Usually transmitted by the bite of infected fleas. Pneumonic plague may be transmitted by respiratory secretions.	DROPLET PRECAUTIONS for pneumonic plague only. Otherwise STANDARD PRECAUTIONS .	For 24 hours after beginning treatment.	Notify Infection Prevention and Management Unit of all suspected or confirmed cases.	NO



Related Documents

SCGOPHCG Infection Prevention and Management Manual:

- Standard and Transmission-based Precautions Policy
- Healthcare Worker Health and Immunisation Policy
- COVID-19 Policy
- Meningococcal Disease (Invasive) Policy
- Influenza-Like Illness Policy
- Tuberculosis Policy
- Multi-Resistant Organisms Policy
- Respiratory Viruses (Non-Endemic) Policy
- Measles Policy
- Clostridioides (*Clostridium difficile*) Policy
- Creutzfeldt-Jakob Disease (CJD) Risk Assessment and Management Policy

References

- 1 Australian Technical Advisory Group on Immunisation (ATAGI) Australian Immunisation Handbook. Australian Government, Department of Health, Canberra, 2018
- 2 Communicable Diseases network Australia Centre for Disease Control (CDC) CDNA Series of National Guidelines (SoNGs). Australian Centre for Disease Control, Commonwealth of Australia, 2025. Accessed 2026 February 6, <https://www.cdc.gov.au/resources/collections/cdna-series-national-guidelines-songs>

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Standards Applicable:	NSQHS Standard 3 (Version 2): Preventing and controlling Healthcare-associate infection				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

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